

PROJECT PROPOSAL

ON

Organizing Awareness Camps on covid 19 immunity related issues

PROPOSAL SUBMITTED TO:

Ministry of Social Justice and Empowerment,
Room. No. 636, 'A', Wing, Shastri Bhawan,
Dr. Rajendra Prasad Road,
New Delhi - 110001 (India)

PROPOSAL SUBMITTED BY:

Ekatvabharat Foundation

**Address: A-430, Ideal park, Gokul nagar, Katraj
Kondhwa road, Pune 411046**

pune District, Maharashtra,

Mobile: 9272799605

E-mail: - kranti.5217@gmail.com

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Attested Documents to be attached along with the proposal.

Sr. No.	ITEMS	Annexure
1	Details of Project	
2	Registration certificate	
3	Memorandum of Association & Constitution of the Organization	
4	List of members of the Executive Committee	
5	Audited statements & statement of Receipt and Payment for three years	
6	Annual Report	
7	A copy of PAN – NO	
8	Copy of the 12-AA, 80-G	
9	Copy of the FCRA	
10	A copy of Bank Account Pass Book	
12	Details of the Staff Employed	
13	Planning Commission Registered No	
14	District Map	

APPLICATION FORM

FORMAT IN WHICH ORGANIZATION SEEKING FINANCIAL ASSISTANCE TO BE PLACED BEFORE THE GRANT – IN – AID COMMITTEE.

1.	Name of the organization with complete postal address with Telephone No Fax No and E-mail ID	Ekatvabharat Foundation Address: A 430,ideal park,gokul nagar,katraj kondhwa road,pune 411046 pune District, Maharashtra, Mobile: 9272799605 E-mail: - kranti.5217@gmail.com
2.	Whether registered under the Societies Registration Act, 1860 , or any other Act, (to be specified) and the date of registration (please enclose a copy of the Registration Certificate)	Registered Under Society Registration Act clause 21 of 1860 Copy of Registration certificate is attached
3.	Name of the person and his/her designation, nominated/authorized to act on behalf of the organization	Secretary Mras Kranti Pradeep Ilake Ekatvabharat Foundation
4.	Name of the Project President, his telephone number, fax number,	Secretary Mrs Kranti Pradeep Ilake

	mobile number and E-mail ID	Ekatvabharat Foundation 92727299605, kranti.5217@gmail.com
5.	Financial assistance sought with break-up of cost estimates (item-wise)	Rupees sixteen lakhs only sixty nine thousands four hundred only) (INR 1669400/-) Breakup annexed with the project details
6.	Time frame for completion of the project	3 months
7.	If the project is to be undertaken by an individual, details of the organization to which he or she is affiliated	Attached with the records
8.	Is it proposed to receive grant/funds from any other source for the same purpose or activity to which this application pertains? If so, details thereof	Not yet received
9.	Additional information if any	Attached with documents

PROPOSAL DETAILS

Background

An immunization or vaccine-preventable disease is an infectious disease for which an effective preventive vaccine exists. If a person acquires a vaccine-preventable disease and dies from it, the death is considered a vaccine-preventable death. The most common and serious vaccine-preventable diseases tracked by the World Health Organization (WHO) are: diphtheria, Haemophilus influenzae serotype b infection, hepatitis B, measles, meningitis, mumps, pertussis, poliomyelitis, rubella, tetanus, tuberculosis, and yellow fever. The WHO reports licensed vaccines being available to prevent, or contribute to the prevention and control of, 25 vaccine-preventable infections.

In 2012, the World Health Organization estimated that vaccination prevents 2.5 million deaths each year. With 100% immunization, and 100% efficacy of the vaccines, one out of seven deaths among young children could be prevented, mostly in developing countries, making this an important global health issue. Four diseases were responsible for 98% of vaccine-preventable deaths: measles, Haemophilus influenzae serotype b, pertussis, and neonatal tetanus.

The Immunization Surveillance, Assessment and Monitoring program of the WHO monitors and assesses the safety and effectiveness of programs and vaccines at reducing illness and deaths from diseases that could be prevented by vaccines.

Vaccine-preventable deaths are usually caused by a failure to obtain the vaccine in a timely manner. This may be due to financial constraints or to lack of access to the vaccine. A vaccine that is generally recommended may be medically inappropriate for a small number of people due to severe allergies or a damaged immune system. In addition, a vaccine against a given disease may not be recommended for general use in a given country, or may be recommended only to certain populations, such as young children or older adults. Every country makes its own immunization recommendations, based on the diseases that are common in its area and its healthcare priorities. If a

vaccine-preventable disease is uncommon in a country, then residents of that country are unlikely to receive a vaccine against it. For example, residents of Canada and the United States do not routinely receive vaccines against yellow fever, which leaves them vulnerable to infection if travelling to areas where risk of yellow fever is highest (endemic or transitional regions).

The Constitution of India recognizes it as a primary duty for every state to improve public health among their population, providing proper services. In line with this, the 11th Plan (2007-2012) asserted the need to ensure provisioning of primary health services to the poorest. However, many health problems remain in India as well as in Tamil Nadu, where Bless implements its projects.

As per the third National Family Health Survey 2005-2006 (NFHS 3), the infantmortality rate (IMR) and under-5 mortality rate (U5MR) in India were respectively 57 and 74 per thousand live births in these years. These figures had to be drastically reduced so that India could achieve the Millennium Development Goals, for which the deadline is 2015. Indeed, India set the objective to reach a 28‰ live births IMR and a 41‰ live births U5MR. However, as we can observe on the Worldbank Databank, IMR was still high (41‰ live births) last year, as well as the U5MR (53‰). The fulfilment of the goals will probably not be possible for 2015 and will doubtlessly take another several years.

What's more, as per the Worldbank survey, India is part of the 25 lower rankedcountries regarding immunization against DPT (diphtheria, pertussis and etanus) and counts 176 cases of tuberculosis per 100.000 people, which is 25 times more than in Italy or France⁴. Finally, life expectancy at birth stands at 66 years old against 83 in Italy and France.

Justification

Immunisation protects children (and adults) against harmful infections before they come into contact with them in the community.

Immunisation uses the body's natural defence mechanism - the immune response - to build resistance to specific infections. Nine diseases can be prevented by routine childhood immunisation - diphtheria, tetanus, whooping cough, poliomyelitis (polio), measles, mumps, rubella, Haemophilus influenzae type b (Hib) and hepatitis B. All of these diseases can cause serious complications and sometimes death.

Immunisation is given as an injection or, in the case of polio vaccine, taken as drops by mouth. Immunisation helps children stay healthy by preventing serious infections.

Technically 'vaccination' is the term used for giving a vaccine - that is, actually getting the injection or swallowing the drops. 'Immunisation' is the term used for the process of both getting the vaccine and becoming immune to the disease as a result of the vaccine. Most people use the terms 'vaccination' and 'immunisation' interchangeably but their meanings are not exactly the same because immunity follows vaccination in most, but not all, cases. For the purposes of this book, we have always used the term 'immunisation' because this is the expression most commonly used in the community.

All forms of immunisation work in the same way. When someone is injected with, or swallows, a vaccine, their body produces an immune response in the same way it would following exposure to a disease but without the person getting the disease. If the person comes in contact with the disease in the future, the body is able to make an immune response fast enough to prevent the person getting sick.

There are two reasons for immunising everyone in Australia:

- Immunisation is the safest and most effective way of giving protection against the disease. After immunisation, your child is far less likely to catch the disease if there

are cases in the community. The benefit of protection against the disease far outweighs the very small risks of immunisation.

- If enough people in the community are immunised, the infection can no longer be spread from person to person and the disease dies out altogether. This is how smallpox was eliminated from the world, and polio has disappeared from many countries.

Vaccines — which protect against disease by inducing immunity — are widely and routinely administered around the world based on the common-sense principle that it is better to keep people from falling ill than to treat them once they are ill. Suffering, disability, and death are avoided. Immunization averted about two million deaths in 2002. In addition, contagion is reduced, strain on health-care systems is eased, and money is frequently saved that can be used for other health services.

Immunization is a proven tool for controlling and even eradicating disease. An immunization campaign carried out by the World Health Organization (WHO) from 1967 to 1977 eradicated the natural occurrence of smallpox. When the programme began, the disease still threatened 60% of the world's population and killed every fourth victim. Eradication of poliomyelitis is within reach. Since the launch by WHO and its partners of the Global Polio Eradication Initiative in 1988, infections have fallen by 99%, and some five million people have escaped paralysis. Between 1999 and 2003, measles deaths dropped worldwide by almost 40%, and some regions have set a target of eliminating the disease. Maternal and neonatal tetanus will soon be eliminated in 14 of 57 high-risk countries.

New vaccines also have been introduced with significant results, including the first vaccine to help prevent liver cancer, hepatitis B vaccine, which is now routinely given to infants in 77% of WHO's Member States. Rapid progress in the development of new vaccines means protection will be available in the near future against a wider range of serious infectious diseases.

Target

The project proposes to organize ten health camps to be organised in the pune District of Maharashtra for the socially and economically vulnerable people, those who are more concerned by the health problems.

It would mostly target the Dalits and Tribal communities, which constitute a scheduled population that cannot access to medical assistance because of a lack of financial means, awareness an primary healthcare services.

Objectives

- To create awareness on immunization diseases and the need of timely immunization.
- To avoid people from contracting diseases.
- To give knowledge on what they should do to reduce their chances of warding off illnesses.
- To raise health awareness among the poorest.
- To work towards improving the health indicators in pune District significantly.

Preparations& Methodology

For each camp, the first step will be to confirm presence of volunteer doctors to attend to the camps. Then will be the communication phase in order to ensure the efficiency of the camps. That is why we will, in different ways, communicate the event to the inhabitants of the targeted villages. The information will be relayed during the week before the camp thanks to the following means:

- The local channels will inform people about the event, giving them the place and date of the camp.
- Handbills will be distributed giving information about the event in the target villages.
- An auto-rickshaw will drive on the roads and in the villages with a loudspeaker on the day before the camp, in order to remind people
- Posters and displays will be prepared for use during the camps.
- A door to door survey will be conducted to get all the family details so that distribution of the materials won't be difficult and there won't be any duplication.
- Primary schools would be contacted to ensure children's participation.
- Tents and necessary furniture arrangements would be planned to finalize.

Ten camps at identified villages in the district would be organized over a period of three months. Proper documentation and beneficiary records would be compiled to analyse the project indicators post implementation.

Benefits / Outcomes

- Increased health indicators.
- People sensitized to immunization diseases.
- Community member participation in camps.
- Peer educators trained.
- Children to benefit with routine immunizations.

Budget

	Calculation	Amount (INR)
A. Communication		
1. Handbills	300000	300000
2. Local channel banners	300000	300000
3. Auto rickshaw with loud speakers	280000	280000
4. Banners for the camps	250 * 10 Nos.	2500
5. Awareness displays	300000	300000
B. Venue		
1. Tent	3500 * 10	35000
2. Necessary furniture	4500 * 10	45000
3. Water arrangements	300 * 10	3000
C. Basic amenities		
1. Food for the doctors / resource experts – 3	480 * 3 * 10	14400
2. Stationery	2000	2000
3. Audio-visual aids (On rent)	2500 * 10	25000
4. TA & DA for the doctors / resource experts – 3	2000 * 3 * 10	60000
5. Resource kits for the visitors	500 * 500	250000
D. Project Management		
1. Salary to a full time dedicated staff	8000 * 3	24000
2. Communication	3000 * 3	9000

3. Travel	4000 * 3	12000
4. Contingency & Misc	2500 * 3	7500
Total		1669400/-

To,

The Secretary

Ministry of Social Justice and Empowerment,
Room. No. 636, 'A', Wing, Shastri Bhawan,
Dr. Rajendra Prasad Road,
New Delhi - 110001 (India)

Subject: Submission of Project Proposal

Sir,

Greetings from Ekatvabharat Foundation

Please find attached our proposal towards **‘ORGANISING TEN AWARENESS CAMPS ON IMMUNISATION DISEASES IN VILLAGES OF pune DISTRICT OF Maharashtra’**. The budget requested for the project is **Rs. 1669400/- (Rupees sixteen lakhs sixty nine thousands four Hundred only)**.

All necessary documents and enclosures are also being attached with the Application. We shall be really grateful if you could consider this important project in the interest of the community of our region.



Mrs Kranti Pradeep Ilake

Ekatvabharat Foundation